

UPMC Presbyterian

200 Lothrop St, Pittsburgh, PA 15213

DISCHARGE SUMMARY Document #: DIS-001

Patient & Admission

Name	Natasha Wood
Date of Birth	1946-05-06
Phone	878-555-5313
Address	Butler, PA, 16001
MRN	MRN-980841
Admit Date	2026-05-01
Discharge Date	2026-05-07
Attending	Benjamin Horton, MD
Attending NPI	1043321813

Insurance

Primary Payer	Medicare
Primary Member #	9HUQ-09N-1699

Primary Discharge Diagnosis

Acute STEMI, anterior wall

ICD-10: I21.09, I25.10

Hospital Course

The patient presented to the emergency department with acute anterior wall ST-elevation myocardial infarction. She reported sudden onset of severe substernal chest pain radiating to the left arm, accompanied by dyspnea and diaphoresis. Initial electrocardiogram demonstrated ST elevation in the precordial leads V1 through V4 with reciprocal changes. Troponin I was markedly elevated at 8.2 ng/mL. The patient was immediately taken to the cardiac catheterization laboratory where coronary angiography revealed complete occlusion of the left anterior descending artery. Primary percutaneous coronary intervention was performed with successful placement of a drug-eluting stent to the LAD with restoration of TIMI III flow. Transthoracic echocardiography obtained on hospital day two showed anterior wall hypokinesis with a left ventricular ejection fraction of 38 percent, consistent with acute myocardial injury.

During her hospital course, the patient was managed with dual antiplatelet therapy, high-intensity statin, beta-blocker, and ACE inhibitor. She experienced one episode of nonsustained ventricular tachycardia on hospital day three, which was self-limited and did not require intervention. Repeat echocardiography on day five demonstrated mild improvement in wall motion with EF estimated at 42 percent. B-type natriuretic peptide was 385 pg/mL. The patient remained hemodynamically stable without recurrent chest pain or arrhythmia. She tolerated ambulation and participated in cardiac rehabilitation consultation.

The patient is being discharged on hospital day six in stable condition with close outpatient cardiology follow-up scheduled for two weeks. She will continue guideline-directed medical therapy and complete a supervised cardiac rehabilitation program. Repeat echocardiography is planned in three months to reassess ventricular function and guide further management decisions.

Procedures Performed

Coronary angiography	CPT 93458
PCI to LAD with DES	CPT 92928

Medication Changes

Ticagrelor 90 mg BID	STARTED
Metoprolol succinate 50 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	9 days
Urgency Tier	CRITICAL
Clinical Flags	post-PCI dual antiplatelet therapy, recent-revascularization, post-acute-MI

Dictated and electronically signed by:
Benjamin Horton, MD
NPI: 1043321813
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